BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Use the individual prescription and appointment calendar. No universal tumour-marker panel or scan interval is assumed.

Check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Blood and organ safety	Full blood count, kidney and liver tests, glucose and blood pressure; electrocardiogram (ECG) if clinically indicated or if you have a cardiac history. Review potassium, swelling, heart health and all medicines.	Blood count, kidney and liver tests, glucose and blood pressure every 2 weeks for the first 3 treatment months, then every 4 weeks. Potassium may need closer checks if low; assess swelling and cardiac symptoms. ECG if clinically indicated or with cardiac history.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval.
Appointments and medicines	Agree the treatment aim, dose and complete medicines list.	Take abiraterone on an empty stomach: at least 2 hours after food, then wait at least 1 hour before eating. Swallow tablets whole with water. Take prednisolone with food. If a daily dose of either medicine is missed, take the usual dose the next day; do not double it.	Keep a current dose and symptom record.	Never stop prednisolone suddenly or when abiraterone is held unless the team gives a steroid plan. Ask about a steroid alert card and illness instructions. Contact the team if vomiting prevents you keeping tablets down. Abiraterone with prednisolone must not be combined with radium-223.	Keep follow-up and recovery checks even after the last dose.
Cancer-specific and specialist tests	PSA and testosterone help assess cancer response and ADT suppression. High risk here requires at least two of Gleason score 8 or more, at least three bone lesions and measurable spread to an internal organ; your team explains which criteria apply.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval.	Agree the named follow-up team and remaining checks.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00577 v3: official regimen

Tests in your plan

Baseline bloods and tests

WHEN

Full blood count, kidney and liver tests, glucose and blood pressure; ECG if clinically indicated or if you have a cardiac history. Review potassium, swelling, heart health and all medicines.

Checks organ safety and suitability before treatment.

Treatment safety monitoring

WHEN

Blood count, kidney and liver tests, glucose and blood pressure every 2 weeks for the first 3 treatment months, then every 4 weeks. Potassium may need closer checks if low; assess swelling and cardiac symptoms. ECG if clinically indicated or with cardiac history.

Finds changes that may need supportive care, a dose hold or adjustment.

Specialist and cancer tests

If part of your plan

WHEN

PSA and testosterone help assess cancer response and ADT suppression. High risk here requires at least two of Gleason score 8 or more, at least three bone lesions and measurable spread to an internal organ; your team explains which criteria apply.

Distinguishes treatment selection, hormone monitoring and cancer response.

THE WHOLE TREATMENT JOURNEY

Abiraterone and prednisolone: metastatic hormone-sensitive prostate cancer: treatment and checks

Newly diagnosed prostate cancer that has spread, remains hormone-sensitive and meets the NCCP high-risk definition, treated together with ADT.

Abiraterone 1000 mg once daily plus prednisolone 5 mg once daily, continuously while helping and tolerated. Continue your ADT.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Blood and organ safety NCCP	Full blood count, kidney and liver tests, glucose and blood pressure; ECG if clinically indicated or if you have a cardiac history. Review potassium, swelling, heart health and all medicines.	Blood count, kidney and liver tests, glucose and blood pressure every 2 weeks for the first 3 treatment months, then every 4 weeks. Potassium may need closer checks if low; assess swelling and cardiac symptoms. ECG if clinically indicated or with cardiac history.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval.
Appointments and medicines NCCP	Agree the treatment aim, dose and complete medicines list.	Take abiraterone on an empty stomach: at least 2 hours after food, then wait at least 1 hour before eating. Swallow tablets whole with water. Take prednisolone with food. If a daily dose of either medicine is missed, take the usual dose the next day; do not double it.	Keep a current dose and symptom record.	Never stop prednisolone suddenly or when abiraterone is held unless the team gives a steroid plan. Ask about a steroid alert card and illness instructions. Contact the team if vomiting prevents you keeping tablets down. Abiraterone with prednisolone must not be combined with radium-223.	Keep follow-up and recovery checks even after the last dose.

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Cancer-specific and specialist tests NCCP	PSA and testosterone help assess cancer response and ADT suppression. High risk here requires at least two of Gleason score 8 or more, at least three bone lesions and measurable spread to an internal organ; your team explains which criteria apply.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval.	Agree the named follow-up team and remaining checks.

What each check is for

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Treatment and review

Daily tablets plus ongoing ADT

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

Ask / act: Blood count, kidney and liver tests, glucose and blood pressure every 2 weeks for the first 3 treatment months, then every 4 weeks. Potassium may need closer checks if low; assess swelling and cardiac symptoms. ECG if clinically indicated or with cardiac history.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-14 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.51